NCCP 00388 · VERSION 4

Your treatment, explained

Extensive SCLC · Days 1–3 twice daily · NCCP 00388 v4 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Oral etoposide · Days 1–3 twice daily

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Your treatment course
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- My treatment
- My schedule
- Between treatments
- When to call
- Tests and review
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

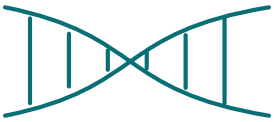
Your chemotherapy

This treatment aims to slow or shrink extensive small-cell lung cancer when intravenous or combination chemotherapy is unsuitable. Benefit varies; your team explains the aim for you.

TOPOISOMERASE-II INHIBITOR CHEMOTHERAPY

Etoposide capsules

Supplied brand may vary



Disrupts DNA copying and repair

Etoposide interferes with topoisomerase II, an enzyme involved in DNA structure and repair. DNA damage can stop cancer cells dividing. Healthy blood-forming and other cells can also be affected. Absorption by mouth varies, so oral and intravenous doses are not interchangeable.

Medicine reaches cells → DNA damage persists → Cell division disrupted

What to watch for

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Your treatment course

REVIEW EACH CYCLE

Chemotherapy course

Cycle Review

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

INDIVIDUAL PLAN

Review and next plan

Review

Your team reviews benefit and side effects and agrees whether to continue, change or stop.

Relative educational plan; your team confirms actual appointments and changes.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My tests and checks

Check	Before treatment	During the cycle	Before next course	After treatment
Safety blood tests	Blood count, kidney and liver profile	Extra tests if needed; oral etoposide regular dates confirmed by team	Blood count, kidney and liver review	Follow abnormal results as advised
Disease assessment	Confirm treatment aim	Symptoms reviewed	Scans or other tests on consultant plan	Agree next treatment or follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00388 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Oral etoposide · Days 1–3 twice daily: treatment and checks

Extensive SCLC · Days 1–3 twice daily

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment days	Between courses	Course review
Treatment NCCP 00388	Confirm capsule strength, number, dosing days, storage and contact numbers with the pharmacist.	Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.	Follow support plan and report symptoms	Review benefit/toxicity; next plan
Safety tests NCCP 00388	Blood count, kidney/liver tests	Review symptoms; extra tests if needed	Blood count, kidney and liver tests before treatment and regularly at team-specified intervals. Scans and follow-up follow the consultant plan.	Follow abnormal results

What each check is for

1

Before each course

Before starting

Safety and plan may change

Ask / act: Team confirms next treatment

Sources: NCCP 00388

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29

YOUR CYCLE, DAY BY DAY

Your 21-day cycle.

Select a day to explore what to watch for. These are possible patterns, not a promise about when symptoms start or stop.

> The full information is available below. Interactive day, phase and date controls require JavaScript; use “Read the whole cycle as text” for the complete timeline.

Your date stays on this page only. It is not saved or added to a QR code.

Medicines on Day 1: Etoposide capsules

On a small screen, swipe the chart sideways. Each day is a keyboard-accessible button.



Prescribed capsules	IV
No planned chemotherapy	Blood-count and symptom recovery varies.

Effects vary; no risk curve or guaranteed recovery day is shown.

DAY 1

Treatment days

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

Your next step: Follow the confirmed prescription; report problems promptly.

The next cycle starts 21 days after this one, if your team confirms it.

> Do not use this page to calculate or change doses.

Read the whole cycle as text

Day 1–3: Treatment days

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review. Follow the confirmed prescription; report problems promptly.

Day 4–21: Between treatments

No planned chemotherapy; continue prescribed support and monitoring. Do not restart until the team confirms the next cycle.

Sources: nccp

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before treatment

Confirm capsule strength, number, dosing days, storage and contact numbers with the pharmacist.



STEP 2

Treatment visit or home course

Take etoposide capsules on an empty stomach using the exact prescribed dose and days. Do not open, chew or crush them. Capsule strengths, quantities and storage follow the supplied pharmacy label. Oral and intravenous doses are not interchangeable.



STEP 3

Bring to review

Bring your medicine list, dose record and questions; report new symptoms before starting another course.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00388 v4 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Sickness and bowel symptoms

Use your prescribed prevention or rescue medicine. Report persistent vomiting, diarrhoea or inability to drink; fever with severe tummy pain needs urgent assessment.

2 Blood tests and review

Blood count, kidney and liver tests are checked before treatment and regularly on dates set by your team for the selected schedule.

3 Pregnancy, fertility and medicines

Discuss fertility and contraception before treatment. Tell the team about pregnancy, breastfeeding, all medicines, supplements and vaccines. Check before changing any medicine.

4 Missed or vomited capsules

Do not double a missed dose or take replacement capsules after vomiting without team advice. Record what happened, contact the oncology pharmacist/team, and follow the next scheduled dose in the agreed plan.

5 Safe handling

Take etoposide capsules on an empty stomach using the exact prescribed dose and days. Do not open, chew or crush them. Capsule strengths, quantities and storage follow the supplied pharmacy label. Oral and intravenous doses are not interchangeable. Keep away from children; ask the pharmacy how to return unused or damaged capsules.

Sources: nccp · smpc · ncis

TREATMENT REFERENCE · 01

My treatment

This treatment aims to slow or shrink extensive small-cell lung cancer when intravenous or combination chemotherapy is unsuitable. Benefit varies; your team explains the aim for you.

How it works

Etoposide interferes with topoisomerase II, an enzyme involved in DNA structure and repair. DNA damage can stop cancer cells dividing. Healthy blood-forming and other cells can also be affected. Absorption by mouth varies, so oral and intravenous doses are not interchangeable.

Sources: nccp · smpc

Evidence and limits

Older studies reported tumour responses with oral etoposide. The evidence is limited, and the different oral schedules cannot be assumed to have the same benefit or risks. A small uncontrolled study of a 21-day oral course does not establish a survival advantage or validate all three NCCP schedules. Its participants had recurrent cancer and substantial treatment toxicity; do not transfer its response percentage to an individual.

Sources: trial

TREATMENT REFERENCE · 02

My schedule

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

Administration

Take etoposide capsules on an empty stomach using the exact prescribed dose and days. Do not open, chew or crush them. Capsule strengths, quantities and storage follow the supplied pharmacy label. Oral and intravenous doses are not interchangeable.

Sources: nccp · smpc

TREATMENT REFERENCE · 03

Between treatments

Keep your prescription and contact plan to hand.

Sickness and bowel symptoms

Use your prescribed prevention or rescue medicine. Report persistent vomiting, diarrhoea or inability to drink; fever with severe tummy pain needs urgent assessment.

Sources: nccp · smpc

Blood tests and review

Blood count, kidney and liver tests are checked before treatment and regularly on dates set by your team for the selected schedule.

Sources: nccp · smpc

Pregnancy, fertility and medicines

Discuss fertility and contraception before treatment. Tell the team about pregnancy, breastfeeding, all medicines, supplements and vaccines. Check before changing any medicine.

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Safe handling

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Sources: nccp · smpc

TREATMENT REFERENCE · 04

When to call

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

Blood counts

An infection can be serious even after treatment days have finished. A normal earlier blood test does not rule out a new problem.

Sources: nccp · smpc

TREATMENT REFERENCE · 05

Tests and review

Blood count, kidney and liver tests before treatment and regularly at team-specified intervals. Scans and follow-up follow the consultant plan.

Scans and tumour markers

Your consultant decides which are relevant and when. Do not assume a fixed imaging interval or that tumour markers are required for everyone.

Sources: nccp · smpc

After treatment

Agree follow-up, symptom support and the next plan before the course ends.

Sources: nccp · smpc

Why this treatment may be offered

Older studies reported tumour responses with oral etoposide. The evidence is limited, and the different oral schedules cannot be assumed to have the same benefit or risks.

A small uncontrolled study of a 21-day oral course does not establish a survival advantage or validate all three NCCP schedules. Its participants had recurrent cancer and substantial treatment toxicity; do not transfer its response percentage to an individual.

Sources: trial · nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

Why can treatment be delayed?

Blood results or symptoms may make a delay or dose change safer. Your team confirms when to restart.

Are capsules gentler than an infusion?

No. Oral chemotherapy can cause serious toxicity and needs blood tests and review. Oral and intravenous doses cannot be exchanged.

When will I have scans?

Your consultant sets the timing for your disease and treatment plan. No universal scan or tumour-marker interval is specified here.

> **Try explaining the plan back:** Explain which days you take treatment, when your next blood tests are due and whom you will call tonight if unwell.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00388 v4. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00388 v4 · official regimen
- HPRA Vepesid product information
- NCCP NCIS medical oncology antiemetic guidance V8
- Johnson 1990 oral etoposide phase II study

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6911/388_oral_etoposide_.pdf

https://assets.hpra.ie/products/Human/30580/Licence_PA2239-006-001_27022024163103.pdf

https://healthservice.hse.ie/documents/7180/
Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/2170589/